LIBBY — PATIENT/CAREGIVER TRACK

hgsoc-tuboovarian-stage3c-r0-hrd-pending-idyq

Plain-language summary for patient and caregiver

Generated 2026-08-21

This is decision-support information, not a treatment plan. Please share it with your oncologist before making any decisions based on what's here.

What this page is

This is the patient and caregiver version of a longer report Libby put together about maintenance treatment options for your ovarian cancer. Libby is a research tool. It read your pathology and genetic results, looked at the published studies behind each option, weighed those against what you said matters to you, and ranked the choices. What follows is each option in plain language: what it is, what the studies actually showed, the main side effects, how well it fits your wishes, and where the evidence is thin or still being argued over. None of it replaces a conversation with your own oncologist. Think of it as something to bring to that conversation.

What we know about your cancer

You have high-grade serous carcinoma, the most common type of ovarian cancer, which started in the ovary, fallopian tube, or the lining of the abdomen. It was stage IIIc, meaning it had spread within the abdomen before treatment. You have already been through a lot: chemotherapy (carboplatin and paclitaxel) together with bevacizumab and an investigational immune therapy called GEN-1, and then surgery that removed all of the cancer the surgeon could see. After all of that, your CA-125 blood marker came back to normal, and a blood test that looks for traces of tumor DNA now shows none. Those are both good signs.

Your tumor's molecular testing shapes the options below. The cancer does not carry a BRCA mutation, in either your inherited DNA or the tumor itself. It does look genetically scrambled (what doctors call genomically unstable), but the test that measured this gave a yes-or-no answer rather than the precise score that one of the treatments below requires. The cancer is ER-positive, meaning it carries the estrogen receptor on about 80% of its cells, though it is PR-negative (the related progesterone receptor is absent), which usually means a weaker response to hormone-blocking pills. A separate marker called HER2 came back "low" (a score of 1+ on a 0-to-3 scale). That HER2 result opens up a couple of options worth talking through, but it also comes with some important fine print, which I get to below.

What you told us matters most

You lean toward going after the cancer hard, while staying realistic about side effects at your age. Two side effects are firm lines you do not want to cross: new or worsening nerve damage (numbness, tingling, or pain in the hands and feet, called neuropathy), and severe drops in your blood counts (the kind that bring infections or need transfusions). You would rather take a pill or have a low-burden infusion than be hooked up often. Because you have a history of blood clots and an IVC filter, anything that raises the risk of clots or bleeding needs to be handled carefully. And you are open to clinical trials.

The options

These are listed in the order Libby ranked them. The ranking reflects a mix of how strong the evidence is, how well each option fits your wishes, and how the side effects line up against the lines you drew. It is not a verdict. Your oncologist may reasonably weigh things differently.

One thing to know up front about how this list is built. A first group of options (numbers 1 through 7) was reviewed in detail by an expert panel. A second group (numbers 8 through 11) was found in a later search of newer trials, after that panel review was finished, so those carry less scrutiny and thinner evidence. I flag which is which as you go, and I explain why the later ones sit where they do.

Option 1 — Rucaparib (a maintenance pill)

Rucaparib is a PARP inhibitor, a pill taken twice a day to help keep the cancer from coming back after chemotherapy. In its main study, half the women taking rucaparib went about 20 months before the cancer started growing again, compared with about 9 months for women taking a placebo. That benefit held up even in women whose tumors, like yours, did not have a BRCA mutation, which is what makes this a fair fit for your cancer. One honest limit: the study showed it delays the cancer's return; it has not yet been shown to help people live longer overall.

The main catch is your blood counts. About 3 in 10 women on rucaparib had a serious drop in red blood cells (anemia), and smaller numbers had drops in their infection-fighting white cells. That sits right on top of one of the side effects you said you want to avoid. It does not cause nerve damage, and there was no signal of the dangerous fever-with-low-white-counts problem. It is a pill, which matches what you wanted. If you went this route, it would mean regular blood tests and a clear plan for managing low counts.

Option 2 — Niraparib (a maintenance pill)

This option carries caveats. Here is the tradeoff to weigh.

Niraparib is another PARP inhibitor pill, a very close cousin to rucaparib. In its main study, half the women taking it went about 14 months before the cancer grew again, versus about 8 months on placebo, and like rucaparib it works without needing a BRCA mutation. There is also some early laboratory evidence that niraparib may get into tumors like yours especially well, which is a point in its favor, though that evidence is thin. As with rucaparib, the benefit shown so far is about delaying the cancer's return, not yet about living longer.

Here is the tradeoff. Niraparib hits your blood counts harder than rucaparib does. Roughly 3 in 10 women had a serious drop in platelets (the cells that help blood clot), and about the same fraction had serious anemia. Your white count is already on the low side to begin with, which leaves less cushion. This is exactly the kind of side effect you said you did not want. It is not a reason to rule the drug out, but it is the reason it sits behind rucaparib here rather than ahead of it. If you and your team wanted to use niraparib, the sensible path is a starting dose tailored to your weight and platelet count, with a written plan for managing low counts. With that plan in hand, the concern is largely manageable.

Option 3 — Bevacizumab (continuing the IV drug you are already on)

Bevacizumab is an antibody given by IV that works by cutting off the blood supply tumors use to grow. You are already on it, so continuing it adds nothing new to learn. In its main study, half the women went about 14 months before the cancer grew again, versus about 10 months without it. Like the PARP pills, it has been shown to delay the cancer's return but not to help people live longer overall, so on that front it is a smaller bet than the numbers above.

What makes it stand out is that it clears both of your firm lines. It does not cause nerve damage, and it does not crater your blood counts. The side effects to watch are high blood pressure, which showed up in about 1 in 4 women and is usually controllable with medication, and rare but serious problems like a tear in the bowel wall (around 1 in 40). Because it can raise the risk of clots and bleeding, your clot history and IVC filter mean this needs careful monitoring rather than being a dealbreaker. The open question is whether continuing bevacizumab on its own is enough, or whether a PARP pill should be added once the genetic score below is sorted out.

Option 4 — GEN-1 / IMNN-001 (the investigational immune therapy you have been receiving)

GEN-1 is an experimental treatment that puts a gene for an immune-signaling protein (IL-12) directly into the abdomen to rally the immune system against the cancer. You have already been on it, and it was part of the treatment that got you to a clean surgery. In its study, half the women went about 15 months before the cancer grew again, versus about 12 months without it, and there was an encouraging early hint that it might help women live longer. I want to be straight with you about that hint: it comes from a company summary and a conference presentation, not yet a full published paper, so I can't give you a precise measure of the benefit here without more solid data, and there is no published table of the serious side effects.

It is given into the abdomen rather than as a pill, so it does not fit your preference for low-burden treatment. It has not triggered any of the side effects on your do-not-want list so far. Because it is still investigational, staying on it depends on the trial structure. The argument for it is mostly continuity: it is the regimen that helped get you here, and you are open to trials.

Option 5 — Letrozole (a hormone-blocking pill, with a trial you could join)

Letrozole is a once-a-day pill that lowers estrogen, used because your cancer is ER-positive. It is the gentlest option on this list. It does not touch your blood counts and does not cause nerve damage, which fits your wishes and your age well. And there is a genuine maintenance trial you could enrol on right now at this exact point in your treatment: a randomized study testing hormone-blocking maintenance in women in your situation. That makes letrozole a real maintenance option, not just a mild afterthought, and it fits your openness to trials.

Here is where I have to temper it honestly. That trial has not reported its results yet, so we do not know how much it helps; the hoped-for benefit is a design assumption, not an observed outcome. The published hormone-therapy studies in ovarian cancer are modest. The largest one, using a drug in the same family, found benefit in only about a third of women and a short time to the cancer growing again, and, importantly, it found that how strongly a tumor reads for estrogen did not predict who benefited. So your strong-looking 80% estrogen reading is a rough signal, not a reliable one, and your cancer being PR-negative points to a more modest effect still. An older look-back study of letrozole itself was more encouraging (about 60 out of 100 women free of recurrence at two years versus about 39 out of 100), but a look-back study cannot prove the pill caused the difference the way a proper trial can. Because the estrogen reading here is on the weaker side as a predictor, a repeat or confirmatory look at those markers could sharpen the picture. Bottom line: this is the lowest-side-effect choice, now backed by a real trial you could join, but the size of the benefit is genuinely uncertain and probably modest.

Option 6 — Olaparib plus bevacizumab (a pill-plus-IV combination, only if a genetic test qualifies you)

This option carries caveats, and it depends on a test result you do not have yet. Olaparib is a PARP inhibitor pill; combined with the bevacizumab you are already on, it was the most powerful option on this whole list in its study, but only for a specific group of women. In that group, half went about 37 months before the cancer grew again, versus about 18 months. The important fine print: that big number came from a subgroup of the study, not everyone in it, and to qualify you need a specific validated test showing your tumor is genetically unstable above a set cutoff. The simpler yes-or-no instability result you already have does not meet that bar. If that test comes back below the cutoff, this combination is off the table, and the other options on this page still stand. If it comes back above the cutoff, this jumps up the list as a serious contender.

The other caveat is side effects, and this one is real. The combination is the hardest-hitting option here: more than half the women had a serious side effect of some kind, including high blood pressure in about 1 in 5 and serious anemia in roughly 1 in 6. For you specifically, it stacks two of your concerns at once: the blood-count drops you want to avoid, and the clot-and-bleeding caution from your history. If this option becomes available, it would need a concrete plan for your blood pressure, your clotting risk, and your blood counts before it makes sense.

Option 7 — Trastuzumab deruxtecan, a HER2 drug (investigational for your cancer)

This option carries caveats, and it is not approved for your type of cancer at your HER2 level. This is where the HER2 result comes in. Trastuzumab deruxtecan is a drug that homes in on the HER2 marker and delivers chemotherapy straight to the cancer cell. It is approved and works well in breast cancer, including breast cancers with low HER2 like yours. The problem is that the approval for ovarian and other cancers requires a high HER2 level (a score of 3+), and your tumor is HER2-low (1+). That means there is no direct evidence in ovarian cancer at your HER2 level. Everything we can say about it for you is borrowed from breast cancer studies plus the way the drug is built to work. In those breast studies, women whose cancers had hormone receptors like yours went roughly twice as long before the cancer grew again. Whether that carries over to ovarian cancer at a low HER2 level is genuinely unknown.

A few things to be clear about. Your HER2 reads "low" (1+), which is a weaker, less certain signal that a HER2 drug will help than a "high" (3+) result would be, so this is a more uncertain option than a strong HER2 reading would make it. Because it is not approved for this situation, it is not standard care: getting it would mean a clinical trial or a special off-label request your oncologist would have to arrange. It is given by IV, not as a pill. And it carries a serious lung side effect, inflammation of the lung tissue, that affected about 1 in 9 women in the breast studies and has in rare cases been fatal, so anyone on it needs regular chest scans. On the plus side, it does not cause nerve damage or the severe fever-with-low-counts problem, so it does not cross either of your firm lines. One more thing worth knowing: your HER2-low result rests on a single tissue sample, and the lab that did the genetic testing could not check HER2 at all because there was not enough material. The 1+ versus 0 call is the hardest one for pathologists to agree on. So a sensible first step before taking any HER2 drug seriously would be to re-stain the tumor and confirm the HER2 result. This is most realistic as something to keep in your back pocket for later, if the cancer returns, rather than a treatment to start now.

Option 8 — Trastuzumab deruxtecan plus olaparib (a HER2 drug paired with a PARP pill, for a possible future relapse)

This option carries caveats. It was also found in the later trial search, after the expert panel had finished, so no panel weighed in on it, and its evidence is thin. This pairs the same HER2 drug from Option 7 with a PARP pill, and there is an actual trial recruiting that would accept a tumor like yours if the cancer comes back and becomes resistant to platinum chemotherapy. In an early study, a bit over half the patients responded, which is genuinely encouraging. But that comes from a small, early trial reported at a conference rather than a full published paper, and it mixed different HER2 levels together.

The same HER2 caution from Option 7 applies here and matters even more: your HER2 reads "low" (1+), a weaker and less certain signal that a HER2 drug will help than a high result, so read that encouraging response rate with that hedge in mind. The bigger practical problem is side effects. Adding the PARP pill to the HER2 drug stacks up the blood-count toxicity: in the early trial, some dose groups had to be stopped for dangerously low counts, and even the tolerable one saw serious anemia in about 1 in 4. That lands squarely on the side effect you want to avoid, and your already-low white count leaves little room. It is also given partly by IV. This is realistically a possible option only later, if the cancer returns, and only after re-confirming the HER2 result and putting a blood-count plan in place.

Option 9 — Azenosertib, a DNA-repair-stress drug (for a possible future relapse)

This option carries caveats. It too came from the later trial search, after the panel review, so no panel weighed in on it. Azenosertib is a pill from a newer class that leans on the fact that your cancer's DNA is already scrambled: it pushes cancer cells that are struggling to copy their DNA over the edge. On paper that mechanism lines up with your tumor's biology. There is a trial recruiting for it in ovarian cancer that has come back after platinum chemotherapy.

The honest limits are real. The results reported so far come only from conference presentations, not a full paper, and they were seen in tumors carrying a specific extra marker that has not been tested in your case, so we do not know whether you would fit the group that benefited. Drugs in this class also hit the blood counts hard (a related drug caused serious drops in infection-fighting cells in a majority of patients), which again is the side effect you want to avoid. It is a pill, which fits your preference. This belongs in a conversation about later lines of treatment if the cancer returns, not the decision in front of you now, and it would need that extra marker checked first.

Option 10 — ART0380, a second DNA-repair-stress drug (for a possible future relapse)

This option carries caveats. Like the two above, it surfaced in the later trial search after the panel review, so no panel weighed in on it, and it has the thinnest evidence of the therapeutic options here. ART0380 is another pill that works on the same idea as azenosertib, stressing cancer cells that are already struggling to repair their DNA, which fits your tumor's scrambled biology. There is a trial recruiting that includes ovarian cancer that has returned after platinum chemotherapy.

The catch is that there is no ovarian result to point to yet. The early signals that have been shared were mostly in a different cancer type, and no clear response rate in ovarian cancer has been reported, so I can't give you a meaningful estimate of benefit here. It would also need checking that your particular tumor fits the way the trial selects patients, and the combinations being tested add blood-count side effects that press on the line you drew. It is a pill. Treat this as a possible avenue to raise with your team if and when the cancer returns, not a current choice.

Option 11 — Abemaciclib plus a hormone-blocking pill (for a possible future relapse)

This option carries caveats. It also came from the later trial search after the panel review, so no panel weighed in on it. This pairs a pill called abemaciclib with a hormone-blocking pill, building on the same estrogen-positive biology as letrozole. It is oral and relatively gentle. There is a trial recruiting for it, though entry depends on a molecular review confirming your tumor qualifies.

The reason it ranks near the bottom is that the published evidence points away from your kind of cancer. In the main ovarian study of this combination, the lasting benefits were seen almost entirely in a slower-growing type of ovarian cancer (low-grade serous), not the high-grade type you have, where the response rate was very low.

There is no result yet from the trial itself. The side effects (some drop in blood counts, and diarrhea from abemaciclib) are milder than most options here but still worth watching. This is a speculative, later-line idea adjacent to letrozole, not a strong bet for your cancer specifically.

Option 12 — Disitamab vedotin, another HER2 drug

This option was considered but not recommended. It is on the page because Libby shows you what was weighed and set aside, not only what was kept. Disitamab vedotin is another HER2-targeting drug, and on paper it could technically accept a tumor like yours. It was set aside for one clear reason: its two most common serious side effects are exactly the two you said you do not want. It causes nerve damage in a large share of people (more than half had some, and close to 1 in 5 had it severely), and it drops the infection-fighting blood counts. Both of those land squarely on your do-not-want list, with no good way around them. There is also almost no evidence for it in ovarian cancer, and the practical ways to get it have dried up. For all those reasons it is not recommended for you.

Questions to ask your oncologist

- If we start a PARP pill like rucaparib or niraparib, what exactly is the plan for watching and managing my blood counts, and at what point would we lower the dose or stop?
- Given my low starting white count and my age, is one PARP pill safer for me than the other, and would a tailored starting dose change that?
- I am already on bevacizumab. Should I simply continue it on its own, or is it worth adding a PARP pill once my genetic test comes back?
- Can we order the validated genomic-instability score, and if it comes back below the cutoff, what does that mean for my choices? (The other options on this page would still apply.)
- Am I a candidate for the hormone-maintenance trial with letrozole, and is it worth repeating the estrogen and progesterone tests to sharpen how likely hormone therapy is to help me?
- Is it worth re-staining my tumor to confirm the HER2 result before we treat that finding as real, given that it rests on a single sample and reads only "low"?
- The HER2 and DNA-repair-stress options are for a possible future relapse. If the cancer comes back, which of those trials would be worth looking into, and how would the lung and blood-count side effects be watched?
- Given my clot history and IVC filter, how would we manage the bleeding and clotting risk with bevacizumab, or with the olaparib-plus-bevacizumab combination?

Sources

The recommendations above draw on the following published studies and clinical-trial records.

PMIDs: 20033066, 22204724, 22204725, 29157627, 30647846, 31328463, 31562799, 31851799, 33109627, 33485453, 35658487, 35665782, 37870536, 37988648, 39282896

Clinical trials: NCT00262847, NCT02477644, NCT02655016, NCT03393884, NCT03522246, NCT04111978, NCT04469764, NCT04585958, NCT04657068, NCT05128825, NCT06003231